

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: A Multicenter, Prospective, Study to Evaluate the Impact of Modifying the Validated Psoriasis Epidemiology Screening Tool (PEST) on the Potential Diagnosis of Psoriatic Arthritis in Adult Patients With Moderate-to-severe Plaque Psoriasis in Canada **Short Title/Acronym:** ScreenX (Modifying PEST for Psoriatic Arthritis Screening) **Protocol Number:** CAIN457ACA06 **NCT Number:** NCT06382051 **Sponsor Name:** Novartis **Investigational Product:** PEST+pictures+2 (Diagnostic Screening Tool) **Phase of Development:** N/A (Observational / Diagnostic Test) **Medical Monitor:** Novartis Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the impact of adding two questions (pertaining to morning stiffness and low back or buttock pain) and pictures to the validated PEST on the potential diagnosis of Psoriatic Arthritis (PsA) in participants with moderate-to-severe plaque Psoriasis (PsO) in Canada who are candidates for biologic disease-modifying antirheumatic drugs (bDMARDs). **Secondary Objectives:** To describe and evaluate the baseline characteristics of patients who score positive versus negative on the standard PEST and the modified PEST+2 tests.

2.2 Background & Rationale Research shows that the standard PEST effectively identifies PsA early in individuals with psoriasis. However, early detection remains an unmet need because PsA often goes unnoticed in patients with plaque psoriasis until joint damage has progressed. This trial aims to determine if an updated version (PEST+pictures+2)—incorporating pictures of symptoms like swollen joints and questions regarding morning stiffness and nighttime pain—can improve early diagnostic screening in real-world dermatology settings.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational / Prospective Cohort **Control Type:** Single-arm (Diagnostic screening evaluation) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target N\$: 502 **Number of Sites:** Multicenter (Canada) **Estimated Study Start:** 23-Jan-2025 **Estimated Primary Completion:** 28-Aug-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Moderate-to-severe plaque PsO patients who are candidates for bDMARDs, as per provincial reimbursement criteria. 2. Adult patients (≥ 18 years) at the time of informed consent signature. 3. Patients able to understand and willing to comply with protocol requirements, instructions, and restrictions. 4. Residents of Canada.

4.2 Exclusion Criteria 1. Patients who have previously screened positive for PsA through PEST or any other screening method. 2. Patients who have been previously diagnosed with PsA and/or are currently followed by a rheumatologist. 3. Patients who have been diagnosed with inflammatory arthritis unrelated to PsA (e.g., rheumatoid arthritis, reactive arthritis, enteropathic arthritis, axial spondyloarthritis). 4. Patients currently treated with a bDMARD for moderate-to-severe plaque PsO or any other medical condition.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Single administration of the modified PEST screening questionnaire (PEST+pictures+2). **Route of Administration:** Questionnaire (Patient-reported outcome measure). **Duration of Treatment:** Up to 66 days (study enrollment duration).

5.2 Concomitant & Prohibited Therapy Permitted: Standard of care topical therapies or conventional synthetic DMARDs for psoriasis. **Prohibited:** Concurrent treatment with any biologic DMARDs (bDMARDs).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Initial Dermatologist Visit	Day 0	Informed Consent, Medical History, Patient fills out the PsA screening questionnaire (PEST+pictures+2).
Rheumatologist Assessment	Day 1 to 65	For patients screening positive: a full PsA diagnosis assessment is performed by a rheumatologist.
End of Study (EOS)	Up to Day 66	Remote/virtual visit conducted by the dermatologist to document the patient's bDMARDs treatment choice and status.

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: The impact of adding two questions (presence of morning stiffness and low back or buttock pain) to PEST (i.e., PEST+2) to improve screening and the potential diagnosis of PsA. **Measurement Method:** Concordance

analysis between the questionnaire screening outcomes (positive/negative) and the formal clinical diagnosis confirmed during the Rheumatologist assessment.

7.2 Secondary & Exploratory Endpoints * Baseline characteristics (age, sex, years since PsO diagnosis, medical history, disease characteristics, height, weight) of positive screening score vs. negative screening score patients for both PEST and PEST+2.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard reporting during the 66-day observation period. Note: as an observational diagnostic study, AEs are monitored primarily in relation to standard-of-care treatments. **Serious Adverse Events (SAEs):** Reported within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Internal monitoring mechanisms managed by Novartis.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Screen (All enrolled participants who complete the baseline questionnaire). **Sample Size**

Justification: $N=502$ appropriately powered to detect statistically significant improvements in diagnostic sensitivity and specificity of the modified PEST versus standard PEST.

Handling of Missing Data: Standard handling methods (e.g., Multiple Imputation) for missing questionnaire items or incomplete rheumatologist assessments.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Centralized and remote monitoring of questionnaire completion rates and subsequent diagnostic follow-up. **Audit Trail:** Secure eCRF locking and data clarification forms via standard quality audit mechanisms.

11. Study Identifiers & Governance

Primary ID: CAIN457ACA06 **Registry Number:** NCT06382051 **Sponsor/Lead Organization:** Novartis **Study Title:** ScreenX (Modifying PEST for Psoriatic Arthritis Screening) **Official Regulatory Title:** A Multicenter, Prospective, Study to Evaluate the Impact of Modifying the Validated Psoriasis Epidemiology Screening Tool (PEST) on the Potential Diagnosis of Psoriatic Arthritis in Adult Patients With Moderate-to-severe Plaque Psoriasis in Canada

12. Clinical Framework (Psoriatic Arthritis / Plaque Psoriasis Specific)

Primary Condition: Moderate-to-severe Plaque Psoriasis (PsO) / Psoriatic Arthritis (PsA) **Diagnosis Threshold:** Positive score on PEST+pictures+2 requiring rheumatologist confirmation **Medical Methodology:** PEST Questionnaire Screening Protocol **Optimization Target:** Early detection of PsA for bDMARD treatment planning

13. Study Procedures & Methodology

Management Pathway: Clinical screening pathway (Dermatologist screening $\rightarrow$ positive screen referral to Rheumatologist) **Education Protocol (HCP):** Site-level training on administering the modified PEST+pictures+2 tool. **Education Protocol (Patient):** In-person questionnaire instructions and visual aid review. **Quality Audit Mechanism:** Tracking diagnostic concordance between screening scores and the actual rheumatologist diagnosis.

14. Observation & Follow-Up Schedule

Total Duration: Up to 66 days per patient **Onsite Visit Cadence:** Initial Dermatologist Visit (Day 0), Rheumatologist Visit (for positive screens) **Remote Monitoring Frequency:** Remote EOS visit up to Day 66 **Checklist Review Interval:** Single baseline evaluation + EOS.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[02-Apr-2026]				
Clinical Operations Lead	[Name]	____	[02-Apr-2026]		Lead		
Statistician	[Name]	____	[02-Apr-2026]				